

1. Administrative & Study Identification

Full Study Title A Phase III, Randomised, Double-Blind, Placebo-Controlled, Study of Durvalumab as Consolidation Therapy in Patients With Locally Advanced, Unresectable NSCLC, Who Have Not Progressed Following Definitive, Platinum-Based Chemoradiation Therapy **Short Title/Acronym** A Study of Durvalumab as Consolidation Therapy in Non-Small Cell Lung Cancer Patients (PACIFIC-5) **Protocol Number** D933YC00001 **NCT Number** NCT03706690 **Posting ID** 179082505906474872 **Sponsor/Company Name** AstraZeneca **Trial Status** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Investigational Product** Durvalumab (MEDI4736) / Placebo **Phase of Development** Phase III **Medical Monitor** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of durvalumab treatment compared with placebo in terms of Progression-Free Survival (PFS) in the Modified Intent-to-Treat (mITT) Set. **Secondary Objectives:** To evaluate Overall Survival (OS), Progression-Free Survival (PFS) in the Intent-to-Treat (ITT) Set, and the Percentage of Participants Alive at 24 Months (OS24).

2.2 Background & Rationale To address the need for optimized consolidation strategies in patients with unresectable Stage III NSCLC. This study was designed to validate and expand upon the practice-changing PACIFIC trial results by assessing durvalumab at a fixed dosing schedule of 1500 mg every 4 weeks (Q4W), incorporating mandatory PD-L1 assessments, and allowing for both concurrent and sequential definitive platinum-based chemoradiation therapy (cCRT or sCRT).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 2:1 (durvalumab to placebo) **Status:** Active, not recruiting

3.2 Planned Enrollment & Duration Target \$N\$: 407 evaluable patients **Number of Sites:** 82 investigational sites across 10 countries **Estimated Study Start:** October 2018 **Estimated Primary Completion:** June 2024 (Data Cutoff)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years. 2. Documented NSCLC and present with locally advanced, unresectable (Stage III) disease. 3. Receipt of concurrent or sequential chemoradiation therapy. 4. No progression following definitive, platinum-based, concurrent or sequential chemoradiation therapy. 5. World Health Organization (WHO) Performance Status (PS) of 0 or 1. 6. No prior exposure to any anti CTLA-4, anti-PD-1, anti-PD-L1, or anti PD L2 antibodies, excluding therapeutic anticancer vaccines. 7. Adequate organ and marrow function required. 8. Life expectancy of at least 12 weeks. 9. Tumor PD-L1 status, with the Ventana SP263 PD-L1 IHC assay determined by a reference laboratory, must be known prior to randomization. 10. Provision of a tumour tissue sample (newly acquired sample $\leq$ 3 months old is preferred, but an archived sample $\leq$ 6 months old is acceptable) in a quantity sufficient to allow for analysis.

4.2 Exclusion Criteria 1. History of allogeneic organ transplantation, or another primary malignancy, or active primary immunodeficiency. 2. Active or prior documented autoimmune or inflammatory disorders. 3. Uncontrolled intercurrent illness that would limit compliance with study requirement, substantially increase risk of incurring AEs, or compromise the ability of the patient to give written informed consent. 4. Active infection including tuberculosis, hepatitis B, hepatitis C (HCV), or human immunodeficiency virus (positive HIV 1/2 antibodies). 5. Mixed small cell and NSCLC histology, sarcomatoid variant. 6. Any unresolved toxicity National Cancer Institute (NCI) Common Terminology Criteria for Adverse Events (CTCAE) Grade $\geq$ 2\$ from the prior chemoradiation therapy. 7. Receipt of live attenuated vaccine within 30 days prior to the first dose of IP. 8. Major surgical procedure (as defined by the Investigator) within 28 days prior to the first dose of IP.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Durvalumab (Trade name: Imfinzi, ATC code: L01FF03) 1500 mg or matching placebo administered every 4 weeks (Q4W). **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Continued until clinical progression/deterioration, confirmed radiological progression, unacceptable toxicity, or consent withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for symptoms and toxicities. **Prohibited:** Concurrent systemic anti-cancer therapy, investigational agents, and live attenuated vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor scans, PD-L1 assessment
Treatment	Every 4 Weeks	IV Durvalumab or Placebo administration, safety & lab assessments
Follow-Up	Weeks 8 to 48	Tumor scans every 8 weeks \$\\pm\$ 1 week up to 48 weeks
End of Study	> 48 Weeks	Tumor scans every 12 weeks \$\\pm\$ 1 week thereafter until confirmed PD

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) (Modified Intent-to-Treat [mITT] Set). **Measurement Method:** Assessed per RECIST 1.1 using Blinded Independent Central Review (BICR). Defined as the time from the date of randomization until the date of objective disease progression (PD) or death (by any cause in the absence of progression) regardless of whether the participant withdrew from therapy or received another anti-cancer therapy prior to progression. PD is defined as at least a 20% increase in the sum of diameters of target lesions (TLs) and an absolute increase of ≥ 5 mm. Median PFS is calculated using the Kaplan-Meier technique.

7.2 Secondary & Exploratory Endpoints 1. Overall Survival (OS) 2. Progression-Free Survival (PFS) (Intent-to-Treat [ITT] Set) 3. Percentage of Participants Alive at 24 Months (OS24)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard clinical and laboratory monitoring prior to each dose, with specific focus on immune-mediated AEs and pneumonitis/radiation pneumonitis. **Serious Adverse Events (SAEs):** Reported within standard 24-hour safety timelines. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) overseeing safety and efficacy signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Modified Intent-to-Treat (mITT) set used for primary efficacy endpoints (PFS). Intent-to-Treat (ITT) set used for secondary efficacy analysis. **Sample Size Justification:** Sized to approximately 407 enrollees to provide adequate power (α and Power standard limits) to detect a statistically significant reduction in the risk of disease progression or death compared to placebo. **Handling of Missing Data:** Standard censoring rules applied for time-to-event endpoints (PFS/OS) when patients

withdrew from therapy or received another anti-cancer therapy prior to progression.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
Monitoring Plan: Routine onsite and remote monitoring to ensure protocol compliance and safety. **Audit Trail:** Utilizing validated Electronic Data Capture (EDC) systems and Blinded Independent Central Review (BICR) to ensure data integrity for tumor scan assessments.

11. Study Identifiers & Governance

Primary ID: {{D933YC00001}} **Registry Number:** {{NCT03706690}}
Posting ID: {{179082505906474872}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Study Title:** {{A Study of Durvalumab as Consolidation Therapy in Non-Small Cell Lung Cancer Patients}}
Official Regulatory Title: {{A Phase III, Randomised, Double-Blind, Placebo-Controlled, Study of Durvalumab as Consolidation Therapy in Patients With Locally Advanced, Unresectable NSCLC, Who Have Not Progressed Following Definitive, Platinum-Based Chemoradiation Therapy}}

12. Clinical Framework (Oncology Specific)

Primary Condition: {{Non-Small Cell Lung Carcinoma / Carcinoma}}
Disease Areas: {{Carcinoma, Non-Small-Cell Lung}} **Diagnosis Threshold:** {{Locally advanced, unresectable Stage III disease}}
Medical Methodology: {{Consolidation immunotherapy following cCRT or sCRT}} **Optimization Target:** {{Improvement in Progression-Free Survival (PFS)}}

13. Study Procedures & Methodology

Management Pathway: {{Consolidation Therapy Administration}}
Education Protocol (HCP): {{Identification and management of immune-mediated adverse events (imAEs) and pneumonitis}}
Education Protocol (Patient): {{Recognition of respiratory symptoms and standard reporting procedures}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) of imaging data}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to confirmed disease progression (PD) or death}} **Onsite Visit Cadence:** {{Every 4 weeks during treatment phase}} **Remote Monitoring Frequency:** {{Continuous via standard EDC entry}} **Checklist Review Interval:** {{Tumor imaging assessments every 8 weeks, shifting to every 12 weeks post-48 weeks}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				